



1CD-0-3

carcinoma, infiltrating duct, Nos 8500/3
Site: breast, Nos C50.9 for 5/19/11

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copy No. 4

Date

Examination: Histopathological examination

Internal invoice No.

Cost of diagnostic procedure

Examination No

Patient: XXX

PESEL: XXX

Age:

Gender: F

Material: 1. Total organ resection - left breast.

Unit in charge:

Physician in charge:

Material collected on:

Material received on:

Expected time of examination: up to 8 working days

Clinical diagnosis: Cancer of the left breast

Examination performed on:

Macroscopic description:

Left breast sized 16.8 x 13.3 x 4.2 cm removed along with axillary tissues sized 8 x 4 x 3 cm and a skin flap of 17.8 x 9.2 cm.

Tumour sized 4.3 x 2.3 x 2.7 cm found in the upper inner quadrant, 2.4 cm from the lower boundary, 0.6 cm from the base and 1.2 cm from the skin.

Microscopic description:

Carcinoma ductale invasivum NHG2 (3+2+1, 0 mitoses/10 HPF, visual area diameter 0.55 mm).

Reactio lymphocytaria peritumoralis. Mamilla sine laesionibus.

Numerous focuses of carcinoma ductale in situ (DCIS) found within tumour (solid type, with medium nuclear atypia and comedo necrosis, 30% of the tumour).

Invasio carcinomatosa vasorum.

Mamilla sine laesionibus.

Normal parenchyma in the glandular tissue.

Axillary lymph nodes

Metastases carcinomatosa in lymphonodis (No II / XVII).

Histopathological Diagnosis:

Carcinoma ductale invasivum mammae sinistrae. Invasive ductal carcinoma of the left breast.

Metastases carcinomatosa in lymphonodis axillae (No II / XVII). Cancer metastases in axillary lymph nodes (No II/XVII) (NHG2, pT2, pN1a).

Examination performed on:

Compliance validated by:

Results of immunohistochemical examination:

Estrogen receptors found in over 75% of neoplastic cell nuclei. Progesterone receptors found in over 75% of neoplastic cell nuclei. HER2 protein stained with Ventana's Pathway HER-2/neu (4B5) Rabbit Monoclonal Antibody. Negative reaction in invasive cancerous cells (Score = 1+).

Compliance validated by:

Criteria	Yes	No
Diagnosis Discrepancy		X
Primary Tumor site Discrepancy		X
IHPAA Discrepancy		X
Prior Malignancy History		X
Dual/Synchronous Primary		X
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewed by:	5/19/11	